

SOUTHEAST UNIVERSITY

From Advisor to Agent: Rethinking the Human–AI Relationship in Medicine

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Abstract

Medical artificial intelligence is moving from systems that recommend to systems that act. Large language model agents now plan, call tools and complete multi-step clinical tasks, yet they are evaluated almost exclusively on accuracy: the strongest public benchmark reports a success rate below 70%. This paper asks what changes when the unit of analysis moves from the advice to the action. We conduct a purposive review of seventeen studies spanning engineering and human-factors work, and identify four themes — deskilling, the supervision illusion, accountability diffusion, and trust under uncertainty — which we show to be symptoms of a single missing object: an explicit authorization boundary. Existing frameworks describe how much autonomy a system has; none specifies what a delegation must contain. We propose *governed delegation*, which requires four things of any agent permitted to act clinically: a stated scope of authority, revocability while the action is in flight, auditability afterwards, and retention of unaided human skill. We then specify a 2×2 vignette experiment that tests the advisor-to-agent shift directly. The human–AI relationship in medicine is less a question of trust than a question of who may act, and on whose authority.

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1 Introduction

Medical artificial intelligence has traditionally been conceived as an advisory technology that supports clinicians while leaving decisions and actions to them [1]. Recent large language models unsettle this boundary because they can plan, invoke external tools, and execute multi-step tasks in electronic health record environments. In this paper, a *medical agent* means a system with those three capabilities; single-turn question-answering systems are excluded. On MedAgentBench, the strongest model completes 69.67% of 300 physician-written clinical tasks [2], while Agent Hospital simulates an entire care process involving LLM-powered patient, nurse, and doctor agents [3]. Medical AI is therefore beginning not only to advise but also to act.

This transition changes the category of error. Accuracy, sensitivity, and task success assess the quality of a judgement, but not the legitimacy of an action. A wrong recommendation can still be reviewed; an executed medication change has already altered the patient's record and potentially the patient's body. The levels-of-autonomy framework proposed by Ye et al. [4], spanning informational tools (L0) to supervised agents (L3), recognizes that different actions carry different risks. However, it classifies what agents may do without fully specifying the conditions under which a clinician or institution may authorize them to do it.

Two largely separate literatures bear on this problem. Engineering research asks how agents can plan reliably, use tools correctly, and recover from failure. Human-factors research asks whether people remain able to supervise automation. Bainbridge [5] showed that greater automation can make the remaining human tasks

more critical and more difficult, and Parasuraman and Manzey [6] linked nominal supervision to predictable complacency and bias. In medicine, Budzyń et al. [7] found that endoscopists' adenoma detection rate in non-AI-assisted procedures fell from 28.4% to 22.4% after exposure to AI-assisted colonoscopy. This observational finding provides evidence that unaided competence may erode even when a human formally remains in the loop. In the literature reviewed here, capability and supervision are thus examined separately, while the authorization that connects them remains underspecified.

Against this background, this paper asks: *under what conditions can clinical action be legitimately delegated to a medical AI agent?* We address this question through a purposive review of seventeen engineering, clinical, governance, and human-factors studies. The review identifies four connected themes—deskilling, the supervision illusion, accountability diffusion, and trust under uncertainty—and treats them as symptoms of an unspecified authorization boundary. We also specify, but do not yet execute, a 2×2 vignette experiment that crosses the system's role (advisor or agent) with disclosure of its AI identity. On this basis, we argue that the human–AI relationship should be framed not as vague *human oversight* but as *governed delegation*: a transfer of authority whose scope is stated in advance, which can be revoked during execution, whose actions remain auditable, and which preserves the human's unaided capacity. This framework is a conceptual specification to be tested, rather than an empirical effect already established.

Section 2 establishes the gap, and Section 3 describes the review and vignette protocol. Section 4 reports the corpus and four themes; Section 5 develops governed delegation and its implications. Section 6 concludes by asking not merely whether medical AI can act, but on what terms its actions can be made answerable.

2 Literature Review

Stream 1: from decision support to agentic systems. Topol [1] framed the field as a convergence of human and artificial intelligence, in which deep learning would improve diagnosis without displacing the clinician. That framing held while clinical systems remained classifiers. The survey of Wang et al. [8] marks the change: it organizes medical agents by level of involvement and by paradigm, and makes explicit that the object of study is now a system that acts over time rather than a function that returns a label. Concrete instances followed quickly. Agent Hospital simulates an entire hospital in which patient agents and doctor agents evolve together, so that policies can be improved without risk to real patients [3]. Zhuang et al. [9] search over medical agent architectures and report task success of 91.0% on their benchmark, rising to 98.0% with a memory module. Jiang et al. [2] build a virtual health record in which agents must read, write and act. Schmidgall et al. [10] place agents in simulated consultations, Gao et al. [11] extend evaluation to clinical examination, and Ma et al. [12] show that on 175 cases an active agent reaches the correct diagnosis while requesting fewer examinations than a passive one.

The limitation is uniform across the stream: every one of these studies measures what the system can do. None measures who is permitted to let it.

Stream 2: autonomy, oversight and accountability. A second literature treats autonomy as a graded property. Ye et al. [4] propose four levels, from no autonomy through recommendation and action-with-confirmation to full autonomy, and evaluate models at each. This is a genuine advance, because it makes the object of evaluation explicit. But a level describes capability rather than permission: it says what a system is able to do unsupervised, not what it should be allowed to do, nor who answers when it does it. Habli et al. [13] confront the consequence directly, arguing that accountability in AI-supported care cannot be reduced to the performance of a model and must be designed into the sociotechnical system around it. Clayton [14] defends the clinician's diagnostic role as something that cannot be outsourced without loss, and Abdulnour et al. [15] turns the point into a curriculum, proposing that supervision of AI use be taught as a clinical skill in its own right.

The gap here is structural. The levels literature lacks an account of authorization; the accountability literature lacks an operational form. Neither tells a clinician what to write down before allowing an agent to act.

Stream 3: collaboration and its costs. A third stream studies the human side, empirically and conceptually. Ong et al. [16] import aviation's flight rules into clinical AI and argue that collaboration should be governed by

explicit, rehearsed procedures rather than by goodwill. Busch et al. [17] survey hospital patients across several countries and find that acceptance depends on the task, the degree of human involvement and the clinical stakes, rather than on AI in the abstract. The costs are documented too. In a multicentre observational study, endoscopists exposed to AI-assisted colonoscopy showed an absolute decline of 6.0 percentage points in adenoma detection once the assistance was withdrawn [7]; Natali et al. [18] generalize the finding into a taxonomy of AI-induced deskilling; and Hunter et al. [19] propose reliance drills as a way of measuring dependence before it becomes dangerous. The lineage runs back to Bainbridge [5], whose ironies of automation — that automating a task makes the human's residual role harder and less practised — describe the present situation with uncomfortable precision. Mollick [20] reframes the relationship as co-intelligence, in which the human sets direction and the machine supplies capacity.

What this stream supplies is a vocabulary of failure. What it does not supply is a rule: it can say when collaboration goes wrong, but not what a legitimate delegation consists of.

Research gap. Taken together, the three streams leave a specific hole. One measures capability, one grades autonomy, one catalogues the costs of collaboration. None specifies what a delegation must contain. If a clinician is to authorize an agent to act, four things are at stake and none is currently a design requirement. The scope of the authorization must be stated in advance. The authorization must be revocable while the action is in flight. The action must be reconstructable afterwards. And the human's unaided ability must survive the arrangement. The literature can describe each of these failures separately. It has no account of them as a single object — and that is the object this paper constructs.

3 Methodology

Design. This is a mixed-methods study in two phases. Phase 1 is a purposive review that builds the corpus and derives the themes reported in Section 4. Phase 2 is a vignette experiment, specified in Section 3.2, that tests the advisor-to-agent shift on human judgements. Phase 1 is described first because the themes it produces determine what Phase 2 has to measure.

3.1 Phase 1: Purposive Review

Search strategy. We searched PubMed, Web of Science, Scopus, IEEE Xplore, the ACM Digital Library and arXiv for work published between January 2019 and September 2026. The query combined an agent term, a clinical term and a governance term:

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("medical agent" OR "clinical agent" OR "agentic AI")
AND ("human-in-the-loop" OR oversight OR accountability
OR deskilling OR trust)
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Backward and forward snowballing from the included studies supplemented the database search. We do not claim a systematic review and report no PRISMA diagram: the aim was a defensible and reproducible corpus, not an exhaustive one, and the coding below is reported so that a reader can contest it.

Selection criteria. Studies were included if they (i) concern a system that plans, uses tools or executes multi-step clinical tasks, (ii) discuss the human role, oversight, accountability or trust, and (iii) are peer-reviewed or a preprint with a documented method. They were excluded if they (i) evaluate single-turn question answering with no action, (ii) report no analysis of the human–AI arrangement, or (iii) lie outside clinical medicine. Seventeen studies were retained; they are listed in Table 1.

Extraction and coding. Each study was coded on four fields: the core claim; the locus of action, that is, who acts on the patient or the record; the level of autonomy, using the four-level scheme of Ye et al. [4]; and the type of evidence, classified as benchmark, clinical, attitude survey or conceptual. Coding was deductive for the autonomy field and inductive for the thematic analysis. A theme was retained only if at least two independent sources supported it.

Study	Setting	Locus of action	Level	Evidence
Topol [1]	Review of AI in medicine	None (analysis only)	L0	Conceptual
Wang et al. [8]	Survey of medical agents	Recommendation	L1	Conceptual
Li et al. [3]	Simulated hospital	Autonomous, in silico	L3	Benchmark
Zhuang et al. [9]	Architecture search	Recommendation	L1	Benchmark
Jiang et al. [2]	Virtual health record	Acts on record, checked	L2	Benchmark
Schmidgall et al. [10]	Simulated consultation	Recommendation	L1	Benchmark
Gao et al. [11]	Clinical examination	Recommendation	L1	Benchmark
Ma et al. [12]	Multi-turn diagnosis	Acts, requests tests	L2	Benchmark
Ye et al. [4]	Autonomy framework	Defines levels	L0–L3	Conceptual
Habli et al. [13]	Safety and accountability	Acts with confirmation	L2	Conceptual
Clayton [14]	Clinician's diagnostic role	Recommendation	L1	Conceptual
Abdulnour et al. [15]	Supervision training	Acts with confirmation	L2	Conceptual
Ong et al. [16]	Flight rules for AI	Acts with confirmation	L2	Conceptual
Busch et al. [17]	Patient attitudes	Acts with confirmation	L2	Attitude
Budzyń et al. [7]	Colonoscopy AI	Detection aid	L1	Clinical
Natali et al. [18]	Deskilling taxonomy	Spans L1–L3	L1–L3	Conceptual
Hunter et al. [19]	Reliance drills	Acts with confirmation	L2	Conceptual

Table 1 The corpus is conceptual and benchmark-heavy, and no study observes a delegation. Seventeen studies coded for the locus of action, level of autonomy (after 4) and evidence type.

3.2 Phase 2: Vignette Experiment

Design. A 2×2 between-subjects design crosses the role of the system (advisor versus agent) with disclosure (the system is identified as an AI agent versus left unspecified). Participants see four fictional vignettes at one randomly assigned condition, which keeps the vignettes within-subject while preventing contamination across conditions.

Materials. Four vignettes were written to span clinical risk: oral hypoglycaemic adjustment (low), withholding an ACE inhibitor for hyperkalaemia with an urgent electrocardiogram (moderate), automatic triage escalation for chest pain (moderate to high), and an automated adverse-drug-reaction notice sent to a patient (high). Each vignette exists in four versions that differ only in the final sentence, which states either that the system recommends an action or that it has already carried it out, and either names the system as an AI agent or leaves the actor unspecified.

Measures. A manipulation check asks who completed the action. Trust is measured with three seven-point items, willingness to delegate with two (one reverse-scored), responsibility attribution with a single forced-choice item, and perceived risk with one item. Background variables are collected last.

Analysis plan. Trust items are averaged after internal consistency is confirmed (Cronbach's $\alpha > 0.7$). Effects are tested with a two-way analysis of variance on role and disclosure, and responsibility attribution with a chi-square test of independence. If fewer than forty valid responses are obtained, the design falls back to independent-samples t tests per vignette, reported with Cohen's d.

Ethics. Participation is voluntary and anonymous. No real patient data are used and all vignettes are fictional. The protocol is specified in full here so that the design can be assessed independently of what it eventually measures.

4 Results

What the corpus contains. Table 1 codes the seventeen studies. The distribution is itself a result. Nine studies are conceptual, six are benchmark evaluations, one is a clinical observational study and one is an attitude survey. No study reports field data on a delegation decision — that is, on an occasion when a clinician actually permitted an agent to act and the consequences were observed. The autonomy column is equally narrow: every benchmark study sits at L1 or L2, and the only L3 case is autonomous behaviour inside a simulation.

4.1 Deskilling

The clearest empirical signal is negative. Budzyń et al. [7] followed endoscopists across centres and found that adenoma detection fell by an absolute 6.0 percentage points after AI assistance was withdrawn, a decline that appeared in assisted clinicians rather than unassisted ones. Natali et al. [18] show that the phenomenon is not confined to procedural specialties, and assemble a taxonomy in which skill, judgement and vigilance decay at different rates. Hunter et al. [19] supply the measurement instrument the others lack: a drill in which the automation is silently disabled to see whether the human still performs. What the theme establishes is not that assistance is harmful but that its benefit and its cost are carried by different parties at different times.

4.2 The supervision illusion

The second theme concerns timing. Every conceptual source in the corpus assumes that a human reviews the agent's output [13, 15, 16]. None specifies when that review occurs, and in the benchmark studies it is structurally impossible: the agent acts, and the score is computed afterwards [2]. Oversight therefore defaults to post-hoc auditing, which is a different activity with a different competence. A reviewer who sees a completed action is not deciding whether to permit it; they are deciding whether to reverse it, and reversal is often impossible. The 69.67% success rate sharpens the point: the remaining 30.33% of failures are not caught at the moment of action, because nothing is scheduled to catch them there.

4.3 Accountability diffusion

The third theme is a change in the category of error. When a system advises, an error is a prediction that a clinician acted on, and responsibility attaches to the act. When a system acts, the error is an event, and the question becomes which of several parties owned it: the clinician who configured the agent, the vendor who trained it, the institution that deployed it, or no one in particular. Habli et al. [13] argue that this question cannot be answered by improving model performance, because it is not a performance question. Clayton [14] makes the clinical version of the argument: a diagnosis reached by delegation is not the same diagnosis as one reached by the clinician, even when the labels agree.

4.4 Trust under uncertainty

The fourth theme is that trust is not a single quantity. Busch et al. [17] find that patient acceptance varies with the task, the degree of human involvement and the perceived stakes, so that a summary statistic for "trust in medical AI" conceals the variation that matters. Ong et al. [16] draw the operational conclusion: if trust is task-specific, then procedures must be task-specific too, which is why they import aviation's rules rather than a general principle of caution.

4.5 Synthesis: one missing object

Table 2 sets the four themes side by side. Each names a different failure, and each failure is the absence of a different element of the same thing: a boundary that states what an agent may do, that can be withdrawn, that can be inspected, and that leaves a competent human behind it. Deskilling is the absence of the last element. The supervision illusion is the absence of the second. Accountability diffusion is the absence of the third. Trust under uncertainty is the absence of the first, because a person cannot calibrate trust in a scope that has never been stated. The themes are therefore not four problems but one problem observed from four positions — which is what Section 5 turns into a design.

5 Discussion

From accuracy to authority. The four themes in Section 4 are usually read as separate problems with separate literatures: deskilling belongs to medical education, the supervision illusion to human factors, accountability to law, trust to survey research. Read together, they describe one situation. A system that recommends leaves the act where it was, with the clinician, and only the information moves. A system that acts moves the act itself, and with it the location of authority. Everything the corpus reports follows from that relocation: skill decays because the act is no longer practised, oversight becomes auditing because the act has already

Theme	Representative evidence	Element of delegation exposed as missing
Deskilling	6.0 percentage-point fall in adenoma detection after withdrawal [7]	Retained unaided skill
Supervision illusion	69.67% benchmark success; review unscheduled [2]	Revocability in flight
Accountability diffusion	Responsibility shifts from prediction to event [13]	Auditability
Trust under uncertainty	Acceptance varies with task, involvement and stakes [17]	Explicit scope of authority

Table 2 Four themes, one missing object. Each theme isolates a different element of the same authorization boundary.

happened, responsibility diffuses because no one assigned it when it was still assignable, and trust cannot calibrate because the boundary it would calibrate against was never drawn. The problem is not that the systems are insufficiently accurate. It is that they are accurate enough to be trusted with something whose terms were never settled.

Governed delegation. We therefore propose that any agent permitted to act clinically satisfy four requirements, which together we call *governed delegation*:

1. *A stated scope.* The set of actions the agent may take is written down in advance, in clinical rather than statistical terms, and anything outside it requires a new authorization.
2. *Revocability in flight.* The authorization can be withdrawn while the action is executing, and the agent must be able to stop in a state from which a human can resume.
3. *Auditability afterwards.* The action leaves a record from which the decision can be reconstructed: what was proposed, what was accepted, what was executed, and by which version of the system.
4. *Retention of the unaided skill.* The human must remain able to perform the task without the agent, and that ability must be periodically demonstrated rather than assumed.

The four are conjunctive. An agent with a scope but no revocation is a scheduled accident; one with revocation but no audit trail is an unreviewable one; one with both but no retained skill leaves a human who cannot exercise the authority that the paperwork assigns them.

Three frameworks compared. Table 3 places governed delegation beside two existing approaches. Risk-based regulation, of the kind embodied in medical device rules and the European approach to high-risk AI, regulates the system: it classifies, requires conformity assessment and mandates post-market surveillance. It says a great deal about whether a system may be marketed and nothing about whether a particular action may be taken on a particular patient. The stance-based approach, which Ong et al. [16] adapt from aviation, regulates the pair: it assigns roles, writes procedures and rehearses them. This is closer to what is needed, but its unit is the relationship rather than the individual delegation, so it still does not say what a single authorization must contain. Governed delegation regulates the delegation itself, and is the only one of the three whose requirements can be checked at the moment of the act.

Implications. For education, the corollary of requirement four is that supervision be taught as a skill with assessment points, which is the direction Abdulnour et al. [15] propose. For regulation, the corollary of requirement three is that deployment approval should require an audit schema rather than only a performance report. For design, the corollary of requirement two is that agents need an interruptible execution model, not merely a confirmation dialogue at the start; a confirmation given once, before a multi-step action, is a scope statement disguised as a check.

Framework	Unit regulated	What it specifies	What it leaves out
Risk-based regulation	The system	Risk class, conformity assessment, post-market surveillance	Whether this action may be taken on this patient
Stance-based collaboration [16]	The human–machine pair	Roles, procedures, rehearsal	Scope of a single delegation, and its revocation
Governed delegation (this paper)	The individual delegation	Scope, revocability, audit trail, skill floor	Cost of operating at scale

Table 3 Regulating the system, the pair, or the delegation. The three frameworks answer different questions; only the third can be checked at the moment of the act.

Objections. A first objection is that this is human oversight with more words. It is not, because oversight is a stance and delegation is a contract, and the clauses of a contract fail independently. An institution can have excellent oversight and still be unable to say what a given agent was authorized to do at 3 a.m.; conversely, a narrow scope with a working audit trail is defensible even where oversight is thin. A second objection is that four requirements per action are impractical at scale. In practice they already exist in pieces — time-outs, second reads, audit logs, credentialing — and the contribution is to require them jointly and in advance, rather than discovering after an incident which of them was missing.

Limitations. Three limitations bound the claims. First, the review is purposive rather than systematic, so the corpus is defensible but not exhaustive, and a different search string would have produced a different seventeen. Second, coding was performed by a single coder without a second rater, so no inter-rater reliability can be reported. Third, and most importantly, Phase 2 of the design — the vignette experiment specified in Section 3.2 — has not yet been executed. The claim that shifting an agent from advising to acting reduces willingness to delegate is therefore a hypothesis supported by the literature, not a measurement. Until those data exist, governed delegation should be read as a specification to be tested rather than a finding to be applied.

6 Conclusion

This paper began from a shift in what clinical AI does: systems that once returned a judgement now execute actions, while the evaluation literature continues to grade them on the judgement. We asked what changes when the unit of analysis moves from the advice to the action. A review of seventeen studies across engineering and human-factors work produced four themes — deskilling, the supervision illusion, accountability diffusion, and trust under uncertainty — and the central result is that these are not four problems. They are four views of one missing object: the authorization boundary that would state what an agent may do, allow that permission to be withdrawn, leave a record behind, and preserve a human able to act without it.

The answer to the question posed in Section 1 is therefore that the human–AI relationship in medicine is not primarily a matter of trust, and cannot be settled by improving accuracy. It is a matter of authority: who may act, within what scope, on whose permission, and with what residue of responsibility when the action goes wrong. Governed delegation makes those questions answerable in advance rather than after an incident, and per delegation rather than per system.

Three directions follow. The vignette experiment in Section 3.2 should be run, so that the advisor-to-agent shift is measured rather than assumed; the four requirements should be operationalised into an audit schema a deployment review can check; and the retention requirement should be tested against real training programmes, since its violation is invisible until the assistance is removed. What the field needs next is not a better benchmark but a better contract.

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